PRIOR AUTHORIZATION POLICY

POLICY: Pulmonary – Corticosteroid/Long-Acting Beta$_2$-Agonist Combination Inhalers Prior Authorization Policy

- Advair Diskus® (fluticasone propionate/salmeterol inhalation powder – GlaxoSmithKline, generic [including Wixela Inhub®])
- Advair® HFA (fluticasone propionate/salmeterol inhalation aerosol – GlaxoSmithKline, generic)
- AirDuo® Digihaler® (fluticasone propionate/salmeterol inhalation powder – Teva) [*discontinued*]
- AirDuo® RespiClick® (fluticasone propionate/salmeterol inhalation powder – Teva, generic [brand discontinued])
- Breo® Ellipta® (fluticasone furoate/vilanterol inhalation powder – GlaxoSmithKline, generic)
- Dulera® (mometasone furoate/formoterol fumarate inhalation aerosol – Merck)
- Symbicort® (budesonide/formoterol fumarate inhalation aerosol – AstraZeneca, generic [including Breyna™])

REVIEW DATE: 09/03/2025

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THE FOLLOWING COVERAGE POLICY APPLIES TO HEALTH BENEFIT PLANS ADMINISTERED BY CIGNA COMPANIES. CERTAIN CIGNA COMPANIES AND/OR LINES OF BUSINESS ONLY PROVIDE UTILIZATION REVIEW SERVICES TO CLIENTS AND DO NOT MAKE COVERAGE DETERMINATIONS. REFERENCES TO STANDARD BENEFIT PLAN LANGUAGE AND COVERAGE DETERMINATIONS DO NOT APPLY TO THOSE CLIENTS. COVERAGE POLICIES ARE INTENDED TO PROVIDE GUIDANCE IN INTERPRETING CERTAIN STANDARD BENEFIT PLANS ADMINISTERED BY CIGNA COMPANIES. PLEASE NOTE, THE TERMS OF A CUSTOMER'S PARTICULAR BENEFIT PLAN DOCUMENT [GROUP SERVICE AGREEMENT, EVIDENCE OF COVERAGE, CERTIFICATE OF COVERAGE, SUMMARY PLAN DESCRIPTION (SPD) OR SIMILAR PLAN DOCUMENT] MAY DIFFER SIGNIFICANTLY FROM THE STANDARD BENEFIT PLANS UPON WHICH THESE COVERAGE POLICIES ARE BASED. FOR EXAMPLE, A CUSTOMER'S BENEFIT PLAN DOCUMENT MAY CONTAIN A SPECIFIC EXCLUSION RELATED TO A TOPIC ADDRESSED IN A COVERAGE POLICY. IN THE EVENT OF A CONFLICT, A CUSTOMER'S BENEFIT PLAN DOCUMENT ALWAYS SUPERSEDES THE INFORMATION IN THE COVERAGE POLICIES. IN THE ABSENCE OF A CONTROLLING FEDERAL OR STATE COVERAGE MANDATE, BENEFITS ARE ULTIMATELY DETERMINED BY THE TERMS OF THE APPLICABLE BENEFIT PLAN DOCUMENT. COVERAGE DETERMINATIONS IN EACH SPECIFIC INSTANCE REQUIRE CONSIDERATION OF 1) THE TERMS OF THE APPLICABLE BENEFIT PLAN DOCUMENT IN EFFECT ON THE DATE OF SERVICE; 2) ANY APPLICABLE LAWS/REGULATIONS; 3) ANY RELEVANT COLLATERAL SOURCE MATERIALS INCLUDING COVERAGE POLICIES AND; 4) THE SPECIFIC FACTS OF THE PARTICULAR SITUATION. EACH COVERAGE REQUEST SHOULD BE REVIEWED ON ITS OWN MERITS. MEDICAL DIRECTORS ARE EXPECTED TO EXERCISE CLINICAL JUDGMENT WHERE APPROPRIATE AND HAVE DISCRETION IN MAKING INDIVIDUAL COVERAGE DETERMINATIONS. WHERE COVERAGE FOR CARE OR SERVICES DOES NOT DEPEND ON SPECIFIC CIRCUMSTANCES, REIMBURSEMENT WILL ONLY BE PROVIDED IF A REQUESTED SERVICE(S) IS SUBMITTED IN ACCORDANCE WITH THE RELEVANT CRITERIA OUTLINED IN THE APPLICABLE COVERAGE POLICY, INCLUDING COVERED DIAGNOSIS AND/OR PROCEDURE CODE(S). REIMBURSEMENT IS NOT ALLOWED FOR SERVICES WHEN BILLED FOR CONDITIONS OR DIAGNOSES THAT ARE NOT COVERED UNDER THIS COVERAGE POLICY (SEE "CODING INFORMATION" BELOW). WHEN BILLING, PROVIDERS MUST USE THE MOST APPROPRIATE CODES AS OF THE EFFECTIVE DATE OF THE SUBMISSION. CLAIMS SUBMITTED FOR SERVICES THAT ARE NOT ACCOMPANIED BY COVERED CODE(S) UNDER THE APPLICABLE COVERAGE POLICY WILL BE DENIED AS NOT COVERED. COVERAGE POLICIES RELATE EXCLUSIVELY TO THE ADMINISTRATION OF HEALTH BENEFIT PLANS. COVERAGE POLICIES ARE NOT RECOMMENDATIONS FOR TREATMENT AND SHOULD NEVER BE USED AS TREATMENT GUIDELINES. IN CERTAIN MARKETS, DELEGATED VENDOR GUIDELINES MAY BE USED TO SUPPORT MEDICAL NECESSITY AND OTHER COVERAGE DETERMINATIONS.

CIGNA NATIONAL FORMULARY COVERAGE:

OVERVIEW

The inhaled corticosteroid (ICS)/long-acting beta$_2$-agonist (LABA) combination inhalers are indicated for the treatment of **asthma**.[1-6] Age indications vary by agent. Fluticasone propionate and salmeterol inhalation powder (Advair Diskus, generic [including Wixela Inhub]), Breo Ellipta (authorized generic), and Symbicort (authorized generic) are also indicated for the maintenance treatment of airflow obstruction and reducing exacerbations in patients with chronic obstructive pulmonary disease (COPD), including chronic bronchitis and/or emphysema.[1,3,5] Advair HFA (and authorized generic) and Dulera are not FDA-approved for the treatment of COPD; however, both products have been studied for this use.[2,4,7-9] AirDuo Digihaler [discontinued] and AirDuo RespiClick (and authorized generic [brand discontinued]) also have not specifically been studied in patients with COPD. However, these agents were filed as a New Drug Application under Section 505(b)(2) of the Federal Food, Drug, and Cosmetic Act.[6] This approval pathway relies in part upon evidence not developed by the applicant. In the case of these agents, the literature and safety and effectiveness evidence supporting the approval and use of Advair Diskus (indicated in patients with COPD) are considered part of the evidence supporting the approval and use of the AirDuo products. Of note, another ICS/LABA inhaler, Symbicort Aerosphere® (budesonide/formoterol fumarate inhalation aerosol), was approved in 2023 for the maintenance treatment of patients with COPD. However, the manufacturer currently has no plans to launch this product in the US, and this product is not targeted in this policy.

Guidelines

The 2025 Global Initiative for Chronic Obstructive Lung Disease (GOLD) guidelines for the diagnosis, management, and prevention of COPD support the use of ICS/LABA therapy (in combination with an inhaled long-acting muscarinic antagonist) in select highly symptomatic patients who are at high risk for COPD exacerbations.[10] The ICS/LABAs are also featured prominently in the 2025 Global Initiative for Asthma (GINA) Global Strategy for Asthma Management and Prevention. They are recommended as part of the stepwise treatment algorithm for patients ≥ 6 years of age.[11] European Respiratory Society (ERS) guidelines on the diagnosis and treatment of chronic cough in adults and children (2020) recommend a short-term trial (2 to 4 weeks) of ICS and long-acting bronchodilator (e.g. a LABA) combination in adults with chronic cough and fixed airflow obstruction.[12]

Other Uses with Supportive Evidence

There are also data to support the use of ICS/LABA inhalers in patients with postinfectious cough. Subacute postinfectious cough may have multiple possible underlying etiologies, including asthma.[13,14] The underlying cause of the cough must be determined before making therapeutic decisions. In this situation, ICS/LABA combination therapy may be used as diagnostic empiric therapy in determining the cause of cough (i.e., rule out asthma). When a patient with subacute cough presents with wheezes, rhonchi, or crackles with a normal chest radiograph, it may be a reasonable option to consider therapy with an inhaled bronchodilator and ICS. If cough following an upper respiratory tract infection persists for > 8 weeks, diagnoses other than postinfectious cough should be considered.

POLICY STATEMENT

Prior Authorization is recommended for prescription benefit coverage of the Corticosteroid/Long-Acting Beta$_2$-Agonist Inhalers. The purpose of this policy is to support the use of the corticosteroid/long-acting beta$_2$-agonist combination inhalers in chronic conditions where the products are indicated or their use is appropriate. All approvals are provided for the duration listed below. In cases where the approval is authorized in months, 1 month is equal to 30 days.

- **Advair Diskus® (fluticasone propionate/salmeterol inhalation powder (GlaxoSmithKline, generic [including Wixela Inhub®])**
- **Advair® HFA (fluticasone propionate/salmeterol inhalation aerosol – GlaxoSmithKline, generic)**
- **AirDuo® Digihaler® (fluticasone propionate/salmeterol inhalation powder – Teva) [discontinued]**
- **AirDuo® RespiClick® (fluticasone propionate/salmeterol inhalation powder – Teva, generic [brand discontinued])**
- **Breo(Ellipta (fluticasone furoate/vilanterol inhalation powder – GlaxoSmithKline, generic)**
- **Dulera® (mometasone furoate/formoterol fumarate inhalation aerosol – Merck)**
- **Symbicort® (budesonide/formoterol fumarate inhalation aerosol – AstraZeneca, generic [including Breyna™])**

is(are) covered as medically necessary when the following criteria is(are) met for FDA-approved indication(s) or other uses with supportive evidence (if applicable):

FDA-Approved Indications

1. **Asthma/Reactive Airway Disease.** Approve for 1 year.

2. **Chronic Obstructive Pulmonary Disease.** Approve for 1 year.

3. **Chronic Bronchitis.** Approve for 1 year.

4. **Emphysema.** Approve for 1 year.

Other Uses with Supportive Evidence

5. **Postinfectious Cough.** Approve for 2 months.
   Note: Postinfectious cough is cough that persists after an acute respiratory infection has resolved.

CONDITIONS NOT COVERED

- **Advair Diskus® (fluticasone propionate/salmeterol inhalation powder (GlaxoSmithKline, generic [including Wixela Inhub®])**
- **Advair® HFA (fluticasone propionate/salmeterol inhalation aerosol – GlaxoSmithKline, generic)**
- **AirDuo® Digihaler® (fluticasone propionate/salmeterol inhalation powder – Teva) [discontinued]**
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- **Breo(Ellipta (fluticasone furoate/vilanterol inhalation powder – GlaxoSmithKline, generic)**
- **Dulera® (mometasone furoate/formoterol fumarate inhalation aerosol – Merck)**
- **Symbicort® (budesonide/formoterol fumarate inhalation aerosol – AstraZeneca, generic [including Breyna™])**

is(are) considered not medically necessary for ANY other use(s) including the following (this list may not be all inclusive; criteria will be updated as newly published data are available):

1. **Acute Cough Associated with the Common Cold.**
   Note: This includes symptoms associated with a current rhinovirus infection. There are no data to support the use of ICS/LABA combination therapy in treating this condition. American College of Chest Physicians (ACCP) guidelines for the treatment of acute cough associated with the common cold do not recommend using an ICS or a bronchodilator in treating this condition.[11,15,16]

2. **Chronic Cough due to Gastroesophageal Reflux Disease (GERD).** There are no data to support the use of ICS/LABA combination therapy in treating this condition. The ACCP guidelines for the management of chronic cough due to GERD recommend treatment of the underlying condition and do not mention the use of any inhaled therapies.[17,18]

3. **Acute Cough due to an Acute Respiratory Infection.**
   Note: Examples of an acute respiratory infection are acute bronchitis, sinusitis, influenza, or pneumonia. An acute exacerbation of chronic bronchitis is not the same as acute bronchitis.
   ACCP guidelines for the management of acute cough due to acute bronchitis in immunocompetent adult outpatients do not recommend routine use of inhaled corticosteroids and inhaled beta-agonists.[19] Current evidence is not sufficient to prove that these therapies are safe and effective at reducing the severity and duration of cough in this setting. Bronchodilators are also not a recommended therapeutic option in treating cough associated with acute bacterial sinusitis.[14] Additional ACCP guidelines for the management of acute cough due to suspected pneumonia or influenza state that there is insufficient evidence on the use of nonantibiotic symptomatic therapies such as ICSs or bronchodilators.[16] There

are no data to support the use of ICS/LABA combination therapy in treating these conditions.

4. **Chronic Cough due to Non-Asthmatic Eosinophilic Bronchitis (NAEB).** There are no data to support the use of ICS/LABA combination therapy in treating this condition. Per the guidelines for the management of chronic cough due to NAEB from ACCP, ICSs are the recommended first-line treatment.[13,20] One of the clinical characteristics of NAEB is chronic cough without evidence of variable airflow obstruction or airway hyperresponsiveness. As a result, a beta-agonist bronchodilator would not be expected to be useful in treating this condition.

5. **Chronic Cough due to Bronchiolitis.** The ACCP guidelines do not recommend bronchodilators as a therapeutic option in treating bronchiolitis.[13,21] Use of asthma medications is discouraged unless other evidence of asthma is present. Guidelines from the American Academy of Pediatrics regarding the diagnosis and management of bronchiolitis (2014) also do not recommend that inhaled corticosteroids or bronchodilators be routinely used in the management of bronchiolitis.[22]

6. **Chronic Cough due to Bronchiectasis.** Limited data are available with budesonide/formoterol (foreign formulation of Symbicort) for the treatment of non-cystic fibrosis (CF) bronchiectasis.[23,24] ACCP guidelines note that in patients with bronchiectasis due to CF or other causes, treatment of respiratory infections and airway clearance techniques are the mainstays of management.[25] In patients with airflow obstruction and/or bronchial hyperreactivity (e.g., asthma and/or COPD), bronchodilators may be of benefit.[13,26] However, the ACCP guidelines and the British Thoracic Society (BTS) guidelines do not recommend treatment with ICSs. Bronchiectasis guidelines from the European Respiratory Society also recommend against offering ICSs to adult, adolescent, and patients with bronchiectasis.[27,28] There may be a role for combination ICS/LABA therapy in patients with coexisting asthma or COPD, but there is no evidence to support this therapy in patients without these concomitant conditions.[26-28]

7. **Whooping Cough/Pertussis.** There are no data to support the use of ICS/LABA combination therapy in treating this condition. According to the ACCP guidelines, LABAs and inhaled corticosteroids should not be offered to patients with whooping cough as there is no evidence to suggest benefit.[13] Although short-acting beta-agonists (SABA) [along with other treatments] have been proposed as standard treatment for whooping cough, one review article reported that treatment with the SABA salbutamol resulted in no change in coughing.[29]

8. **Angiotensin-Converting Enzyme (ACE) Inhibitor-Induced Cough.** There are no data to support the use of ICS/LABA combination therapy in treating this condition. Discontinuation of the ACE inhibitor is the only uniformly effective treatment for ACE inhibitor-induced cough. In those patients in whom the ACE inhibitor cannot be discontinued, pharmacologic therapy aimed at suppressing

cough should be attempted. ICSs and beta-agonists are not recommended therapeutic options.[13]

9. **Psychogenic Cough/Habit Cough/Tic Cough.** There are no data to support the use of ICS/LABA combination therapy in treating these conditions. Non-pharmacological therapies, such as behavior modification, hypnosis and psychiatric therapy are the mainstays of treatment.[13,30]

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4. Dulera® inhalation aerosol [prescribing information]. Whitehouse Station, NJ: Merck; June 2025.
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HISTORY

Type of Revision	Summary of Changes	Review Date
Annual Revision	No criteria changes.	08/16/2023
Annual Revision	No criteria changes.	09/11/2024
Annual Revision	No criteria changes.	09/03/2025

APPENDIX A

ICD-10 Codes Automated for Asthma and COPD

ICD-10 Codes	Code Description
J41*	Simple and mucopurulent chronic bronchitis
J41.0	Simple chronic bronchitis
J41.1	Mucopurulent chronic bronchitis
J41.8	Mixed simple and mucopurulent chronic bronchitis
J42	Unspecified chronic bronchitis
J43*	Emphysema
J43.0	Unilateral pulmonary emphysema (MacLeod's syndrome)
J43.1	Panlobular emphysema
J43.2	Centrilobular emphysema
J43.8	Other emphysema
J43.9	Emphysema, unspecified
J44*	Other chronic obstructive pulmonary disease
J44.0	Chronic obstructive pulmonary disease with (acute) lower respiratory infection
J44.1	Chronic obstructive pulmonary disease with (acute) exacerbation
J44.9	Chronic obstructive pulmonary disease, unspecified
J45*	Asthma
J45.2*	Mild intermittent asthma
J45.20	Mild intermittent asthma, uncomplicated
J45.21	Mild intermittent asthma with (acute) exacerbation
J45.22	Mild intermittent asthma with status asthmaticus
J45.3*	Mild persistent asthma
J45.30	Mild persistent asthma, uncomplicated
J45.31	Mild persistent asthma with (acute) exacerbation
J45.32	Mild persistent asthma with status asthmaticus
J45.4*	Moderate persistent asthma
J45.40	Moderate persistent asthma, uncomplicated
J45.41	Moderate persistent asthma with (acute) exacerbation
J45.42	Moderate persistent asthma with status asthmaticus
J45.5*	Severe persistent asthma
J45.50	Severe persistent asthma, uncomplicated
J45.51	Severe persistent asthma with (acute) exacerbation
J45.52	Severe persistent asthma with status asthmaticus
J45.9*	Other and unspecified asthma
J45.90*	Unspecified asthma
J45.901	Unspecified asthma with (acute) exacerbation
J45.902	Unspecified asthma with status asthmaticus
J45.909	Unspecified asthma, uncomplicated
J45.99*	Other asthma
J45.990	Exercise induced bronchospasm
J45.991	Cough variant asthma
J45.998	Other asthma

COPD – Chronic obstructive pulmonary disease; * Indicates the inclusion of subheadings.

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